

SUPERIOR COURT OF CALIFORNIA, CONTRA COSTA COUNTY
MARTINEZ, CA
DEPARTMENT 16
JUDICIAL OFFICER: BENJAMIN T REYES II
HEARING DATE: 03/04/2026

**INSTRUCTIONS FOR CONTESTING TENTATIVE RULING IN
DEPARTMENT 16**

The tentative ruling will become the ruling of the Court unless by 4:00 P.M. of the Court day preceding the hearing, notice is given of an intent to argue the matter. Counsel or self-represented parties must email Department 16 (Dept16@contracosta.courts.ca.gov) to request argument and **must specify, in detail, what provision(s) of the tentative ruling they intend to argue and why**. Counsel or self-represented parties requesting argument must advise all other counsel and self-represented parties by no later than 4:00 P.M. of their decision to argue, and of the issues to be argued. Failure to timely advise the Court and counsel or self-represented parties will preclude any party from arguing the matter. (Pursuant to Local Rule 3.43(2).)

The prevailing party must prepare and e-file a proposed order after hearing in accordance with CRC 3.1312. The order must include appearances. If the tentative ruling becomes the Court's ruling, a copy of the Court's tentative ruling **must be attached to the proposed order** when submitted to the Court for issuance of the order.

ALL APPEARANCES TO ARGUE WILL BE IN PERSON OR BY ZOOM, PROVIDED THAT PROPER NOTIFICATION IS RECEIVED BY THE DEPARTMENT AS PER ABOVE.

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Law & Motion

1. 9:00 AM CASE NUMBER: C23-00098

CASE NAME: CARL PREWITT VS. TEXACO INC.

HEARING ON SUMMARY MOTION

FILED BY: TEXACO INC.

TENTATIVE RULING:

Before the Court is Defendant Texaco Inc.'s Motion for Summary Judgment, or in the alternative, Summary Adjudication and *Sargon* Hearing ("MSJ").

Plaintiffs' unopposed Request for Judicial Notice is **granted**. (Cal. Evid. Code § 452 (c), (h).)

Factual and Procedural Background

This case involves personal injury claims by Plaintiff Carl Prewitt related to his work as an automotive mechanic and service station operator at an Eastwood Texaco in Birmingham, Alabama from 1975-2003. (UMF 1.) While working there, Mr. Prewitt would pump gasoline for customers as well as check

how much gasoline was left in the station's storage tanks, a process referred to as tank gauging or 'sticking.' (UMF 1.) Plaintiffs contend that both activities exposed Mr. Prewitt to gasoline vapors which contain benzene, thereby exposing him to benzene. (UMF 1.)

In February 2019, Mr. Prewitt was diagnosed with the autoimmune disorder called eosinophilic fasciitis ("EF"). (UMF 4.) Mr. Prewitt was diagnosed with Aplastic Anemia ("AA") in September 2019. (UMF 5.) Plaintiffs' complaint alleges Mr. Prewitt's expose to benzene and benzene containing products (i.e. gasoline) caused his AA. (SAC ¶ 9.) The operative Second Amended Complaint includes four causes of action: (1) negligence; (2) strict products liability – design defect, failure to warn; (3) intentional/fraudulent concealment, and (4) loss of consortium.

Texaco does not address each of the above causes of action separately. Instead, Texaco argues that each of the causes of action require Plaintiffs to establish causation – *i.e.* that the exposure to gasoline (containing benzene) caused Mr. Prewitt's AA. The basis of Texaco's argument is that Plaintiffs' expert Dr. Harrison's opinion should be excluded as it is not based on sound, standard, scientific methodology. As the parties do not address the other elements of the causes of action, the Court will focus its analysis on the causation arguments raised by Texaco. The Court will also address Texaco's separate arguments relating to the design/manufacturing defect claims separately.

Standard

"A party may move for summary adjudication as to one or more causes of action within an action ... if the party contends that the cause of action has no merit...." (California Code of Civil Procedure ("CCP") § 437c (f)(1).) A defendant "has met his or her burden of showing that a cause of action has no merit if the party has shown that one or more elements of the cause of action, even if not separately pleaded, cannot be established, or that there is a complete defense to the cause of action." (CCP § 437c (p)(2).)

Once the defendant "has met that burden, the burden shifts to the plaintiff ... to show that a triable issue of one or more material facts exists as to the cause of action or defense thereto." (*Ibid.*) The plaintiff "shall not rely upon the allegations or denials of its pleadings to show that a triable issue of material fact exists but, instead, shall set forth the specific facts showing that a triable issue of material fact exists as to the cause of action or a defense thereto." (*Ibid.*)

"In ruling on the motion, the court must consider all of the evidence and all of the inferences reasonably drawn therefrom, and must view such evidence and such inferences, in the light most favorable to the opposing party." (*Aguilar v. Atlantic Richfield Co.* (2001) 25 Cal.4th 826, 843 (internal citations and quotations omitted); see also, Code of Civ. Proc. §437c(c).)

Causation on Summary Judgment

"Causation is an essential element of a tort action. Defendants are not liable unless their conduct ... was a 'legal cause' of plaintiff's injury. Generally, the burden falls on the plaintiff to establish causation.'" (*Johnson & Johnson Talcum Powder Cases* (2019) 37 Cal.App.5th 292, 323-24 ("*Johnson & Johnson*") quoting *Whiteley v. Philip Morris, Inc.* (2004) 117 Cal.App.4th 635, 696.)

"[I]n a personal injury action causation must be proven within a reasonable medical probability based upon competent expert testimony. Mere possibility alone is insufficient to establish a prima facie case. ... A possible cause only becomes 'probable' when, in the absence of other reasonable causal

explanations, it becomes more likely than not that the injury was a result of its action.” (*Id.* at 324 quoting *Jones v. Ortho Pharmaceutical Corp.* (1985) 163 Cal.App.3d 396, 402-03.)

Expert Testimony

Three provisions of the California Evidence Code govern the admissibility of expert opinion. First, for a person to provide expert testimony, the witness must be qualified as an expert under Evidence Code section 720. Defendant does not argue that Dr. Harrison is not qualified as an expert.

Assuming the expert is qualified to testify concerning the subject matter of the opinions proffered, sections 801 and 802 govern the admissibility of the expert’s testimony. Under those provisions, “the trial court acts as a gatekeeper to exclude expert opinion testimony that is (1) based on matter of a type on which an expert may not reasonably rely, (2) based on reasons unsupported by the material on which the expert relies, or (3) speculative.” (*Sargon Enterprises, Inc. v. University of Southern California* (2012) 55 Cal.4th 747, 771-772.) In exercising its gatekeeping function, the trial court considers whether there is too great an ‘analytical gap’ between the data relied on by the expert and the opinion offered, warranting exclusion of the expert testimony as irrelevant or too speculative. (*Id.* at 771.)

The court simply determines “whether the matter relied on can provide a reasonable basis for the opinion or whether that opinion is based on a leap of logic or conjecture. The court does not resolve scientific controversies. Rather, it conducts a ‘circumscribed inquiry’ to ‘determine whether, as a matter of logic, the studies and other information cited by experts adequately support the conclusion that the expert’s general theory or technique is valid.’” (*Id.* at 772 citation omitted.) “In short, the gatekeeper’s role ‘is to make certain that an expert, whether basing testimony upon professional studies or personal experience, employs in the courtroom the same level of intellectual rigor that characterizes the practice of an expert in the relevant field.’” (*Ibid.* quoting *Kumho Tire Co. v. Carmichael* (1999) 526 U.S. 137, 152.)

Analysis

Plaintiffs’ claims in this case arise from the assertion that Mr. Prewitt’s exposure to gasoline, which contains benzene, over the course of his career caused him to develop aplastic anemia (“AA”) later in life.

It is undisputed that gasoline contains benzene. The dispute between the parties relates to whether gasoline exposure can cause AA, and if so the level of exposure via gasoline to establish such a causation, and the amount of time between the exposure and Mr. Prewitt’s AA diagnosis.

Aplastic Anemia

Aplastic Anemia is not cancer. (UMF 6.) It is a non-malignant disease of the bone marrow failure that is characterized by a significant decrease in all three blood lineages, *i.e.* red blood cells, white blood cells, and platelets (pancytopenia). (D’s Evid. p. 26 - David Pyatt, Ph.D. analysis.) AA can be ‘*de novo*’ or acquired. (*Ibid.*) *De novo* cases result from, for example, immune dysfunction, stem cell defects, or specific genetic abnormalities. (*Ibid.*) Most AA cases are *de novo*. (*Ibid.*) Acquired AA can result from exposure to a wide variety of drugs, viruses, ionizing radiation, and/or chemicals – including benzene. (*Ibid.*) AA is a rare disease with approximately 10 cases per 1,000,000, or roughly 1,200 new cases a

year in the US. (*Id.* at p. 27.)

It has long been known that excessive exposure to benzene can damage the bone marrow and cause AA. (*Ibid.*) The historical studies mostly related to industries that used pure or extremely high concentrations of benzene, such as rotogravure printers and shoe workers. (*Ibid.*) There have been no epidemiologic studies that show an association between occupational exposure to gasoline (which contains low levels of benzene) and AA. (UMF 12.)

There is a separate disease called Acute Myeloid Leukemia (“AML”) which is a form of blood cancer. In a “fairly small percent” of cases of AML the patient had a preexisting case of AA. (Harrison Depo. at 113:19-23.) It is undisputed that Mr. Prewitt does not have AML. (UMF 9.)

Causation

A plaintiff seeking to hold a company liable because its product or a substance it produced and sold allegedly caused plaintiff to be inflicted with a disease or adverse physical condition, the plaintiff has to prove both general and specific causation. General causation means that the product or substance is capable of causing the disease or physical condition at issue. (*Johnson & Johnson*, supra, 37 Cal.App.5th at 330.) Specific causation means that the product or substance caused the disease or condition in the plaintiff. (*Id.* at 331-32.) If the plaintiff fails to provide evidence showing general causation, then one cannot “reliably conclude the toxin caused the *plaintiff’s* disease, even if other known causes were ruled out.” (*Id.* at 330-31 citations omitted.) In other words, if there is no evidence of general causation there can be no specific causation.

Texaco’s Evidence

Texaco submits the reports of two experts in support of its motion: David Pyatt, Ph.D. and Ross Levine, M.D. Dr. Pyatt is a toxicologist with 30 years of experience researching and publishing in the fields of environmental and occupational toxicology, experimental hematology, carcinogenesis and immunology. Dr. Levine is a treating physician and researcher with a focus on patients with myeloid malignancies, which includes clinical experience in assessing and treating AA patients.

Dr. Pyatt provides a detailed review of the issues in this case with numerous citations to publications that were reviewed that provide the basis for his opinions. In summary, his opinions are that (1) occupational exposure to gasoline does not cause or increase the risk of AA; (2) there is no evidence that Mr. Prewitt’s level of exposure to benzene was at a level significant enough to those shown to cause AA; and (3) there is no scientific support for the claim that Mr. Prewitt’s AA was caused by exposures that began 40+ years and ended 15+ years ago.

Dr. Levine reviewed all of Mr. Prewitt’s medical records, as well as the deposition testimony of Dr. Harrison and Dr. Pyatt. He also reviewed the relevant published literature, primarily focusing on the relationship between benzene exposure and the development of aplastic anemia and EF. Dr. Levine provides an in-depth analysis and discussion of the issues in this case and the basis for his opinions. Dr. Levine’s conclusions are that “benzene-induced genotoxicity is not the cause of Mr. Prewitt’s AA,” and that “it is medically likely that his AA was caused by immune-based disruption of his hematopoietic function, and he had multiple immune-based diseases consistent with a propensity for autoimmune based diseases including AA.” (D’s Evid. at p. 439, Levine Report at p. 5.) In other words, Mr. Prewitt’s AA is caused by an autoimmune disease and is not caused by exposure to

gasoline/benzene.

Texaco has met its initial burden to show that Plaintiffs' claims fail due to a lack of causation. As such, Texaco has shifted the burden to Plaintiffs to show that a triable issue of material facts exists. "To meet this burden, the plaintiff must produce admissible evidence." (*Borders Online v. State Bd. of Equalization* (2005) 129 Cal.App.4th 1179, 1188, citations omitted.)

Plaintiffs' Evidence

Plaintiffs' theory of general causation is that the benzene contained in the gasoline Mr. Prewitt was exposed to can cause AA.

Benzene exposure versus gasoline exposure

There have been no epidemiological studies that show an association between occupational exposure to gasoline and AA. (UMF 12.) Plaintiffs 'deny' this assertion without citing any studies that show an association between gasoline exposure and AA. In fact, Dr. Harrison's cited testimony confirms that no such studies exist (or at least have been cited):

Q. Were you able to find epidemiologic literature specific to the association between gasoline exposure and aplastic anemia?

A. Not specifically between gasoline and aplastic anemia. I'm not aware of any – well, let me put it to this this way: There are studies of gasoline attendants that show – people that pump gasoline that show genotoxicity, and I have those cited.

There – and I'm trying to think where there are any studies of gasoline attendants and aplastic anemia. I can't think of any epidemiological studies that – that have been done one way or the other that shed light on that question. (Harrison Depo. at 19:20-20:9.)

Again, when asked if he found any studies linking **gasoline** exposure to AA he noted that he "didn't find anything particularly applicable or useful that that regard." (*Id.* at 36:2-6.)

Instead, Plaintiffs argue that "gasoline and benzene are equally capable of causing genotoxicity, AML and AA." (Resp. to UMF 12.) Plaintiffs fail to provide any support for this statement. Instead, the only studies review and cited by Dr. Harrison relate to benzene exposure, and do not discuss exposure to gasoline. Dr. Harrison also fails to demonstrate why benzene-specific studies can reliably support his conclusion that gasoline can cause AA. As other courts have noted, the "court cannot simply presume that the qualitative toxic and carcinogenic effects of benzene from *any source* are the same." (*Henricksen v. Conoco Phillips Co.* (2009) 605 F.Supp.2d 1142, 1156.) The simple explanation that gasoline contains benzene, and benzene causes certain diseases (such as AA) cannot be the justification for such extrapolation. Benzene and gasoline are physically and toxicologically different. (UMF 15.) "If it is possible to extrapolate from studies of benzene or other benzene-containing products conclusions regarding gasoline, then it will be **incumbent upon Plaintiffs** to explain and demonstrate why the extrapolation is scientifically proper." (*Ibid.* emphasis added.)

The focus of this litigation is the relationship, if any, between exposure to **gasoline** containing benzene as a component and AA. Dr. Harrison fails to make this connection. None of the studies relied upon have concluded that gasoline has the same toxic effect as benzene, and none have shown

that the benzene component of gasoline is capable of causing AA. Dr. Harrison fails to provide a scientifically supported explanation for his conjecture.

This failure becomes starker when added to the fact that Dr. Harrison relies heavily upon studies showing a connection between high levels of benzene exposure causing AML – which Mr. Prewitt does not have.

AA versus AML

It is undisputed that Plaintiff does not have AML. (UMF 9.) Texaco also asserts that there is no evidence that Mr. Prewitt will develop AML. (UMF 10.) Plaintiffs ‘deny’ this assertion (as opposed to disputing it), indicating that Dr. Harrison opines that Mr. Prewitt is at a ‘measurable risk of developing AML.’ (Response to UMF 10.)

In support, Plaintiffs cite portions of Dr. Harrison’s deposition testimony where he testifies that Mr. Prewitt’s AA is permanent and as such, he has a risk of AA “transforming or becoming Acute Myelogenous Leukemia, AML.” (Harrison Depo. 10:13-19.) He notes that Mr. Prewitt (at the time of the deposition) had AA for six years and that there is a “measurable risk of [AA] developing into AML or relapsing.” (*Id.* at 10:24-11:2.) Dr. Harrison considers AA “in the spectrum of blood disorders that can be caused by chemical exposure,” or one “manifestation of several blood problems that are caused by chemical exposure.” (*Id.* at 14:18-15:5.)

Dr. Harrison does not explain what the basis is for his statement that AA can “transform or become” AML. When asked why he believed it was important to rely on epidemiology that was specific to AML (or other diseases that are not AA), he stated:

A. Because we’re talking about a spectrum of diseases, disorders caused by benzene. So benzene causes aplastic anemia. Aplastic anemia, a certain percentage of those cases will become AML. And there are some papers in the benzene and aplastic anemia reference list where that happened – exactly happened, okay, that had been reported by doctors where first somebody had aplastic anemia and they then developed AML.

And there were patients that just went right to AML, didn’t have aplastic anemia first.

...

[In some studies the doctors followed patients and] they say first they had aplastic anemia. Then they developed AML.

So it’s very relevant because it’s a precursor. It’s – it’s on the spectrum.” (*Id.* at 40:3-41:10.)

He goes on to testify that “there’s a certain percentage of aplastic anemia that becomes AML ... there’s no question there’s a certain percentage that became AML.” (*Id.* at 41:17-42:2.) In his report, Dr. Harrison states: “In many cases where benzene has caused AA, the AA is considered a precursor blood disease that can lead directly to acute myelogenous leukemia (AML).” (Def. Evid. p. 153, Report p.3.) There is no citation to evidence supporting this statement.

Q. Would you agree that of all AML cases that are diagnosed, only about 2 percent of them had a preexisting aplastic anemia?

A. I – I don’t know. I would agree that is probably a fairly small percent. (Harrison

Depo. 18:19-23.)

When asked what the utility was of putting the ‘hematopoietic cancers together with noncancers like [AA] and myelofibrosis,” he explained that there was a “common pathway” for chemicals such as benzene to cause genetic damage. (*Id.* at 15:16-16:22.) The genetic damage can “lead to the loss of production of your platelets, white blood cells, red blood cells. It can lead to abnormal early cancer cells.” (*Ibid.*) In other words, the exposure to chemicals (like benzene) can cause genetic damage “that then leads to different blood disorders.” (*Ibid.*)

Despite all the above, Dr. Harrison does not provide any citations to studies or research which actually state that AA ‘transforms’ or ‘becomes’ AML. He does not explain why, even though he agrees that only a ‘fairly small percent’ of patients with AML had preexisting AA, that he believes AA ‘becomes’ or ‘transforms’ into AML. Nor does he explain what factors are relevant to consider in determining when a case of AML will ‘transform’ into AA.

Based on the above, the Court finds that Plaintiffs have failed to produce admissible evidence supporting its burden to show a triable issue of material fact regarding general causation.

Specific Causation

As the Court determined that Plaintiffs have failed to meet its burden regarding general causation, it is not necessary to address the specific causation arguments. (*Johnson & Johnson*, *supra*, 37 Cal.App.5th at 330-31.) If general causation could be presumed, Plaintiffs still failed to present admissible evidence pertinent to the specific causation requirement.

Differential Diagnosis/Etiology

While it is “generally correct that in many (or even most) instances epidemiological studies provide the best evidence of causation,” that is not the only method for establishing causation. (*Davis v. Honeywell Internat. Inc.* (2016) 245 Cal.App.4th 477, 491.) “[T]here is no requirement that a causation expert rely on a specific study or other scientific publication expressing precisely the same conclusion at which the expert has arrived.” (*Gerner v. BNSF Railway Co.* (2024) 98 Cal.App.5th 660, 677, cases cited therein.)

Plaintiffs have not identified any epidemiological studies that discuss the association between gasoline exposure and AA. (UMF 12.) As there were no such studies, Plaintiffs’ expert instead maintains he performed a ‘differential etiology’ or ‘differential diagnosis’ to support his opinion.

“Differential diagnosis, or differential etiology, is a standard scientific technique of identifying the cause of a medical problem by eliminating the likely causes until the most probable one is isolated....” (*Johnson & Johnson Talcum Powder Cases* (2019) 37 Cal.App.5th 292, 307, fn. 9 quoting *Clausen v. M/V Carissa* (9th Cir. 2003) 339 F.3d 1049, 1057-58 (“*J&J Cases*”)) It is “defined as ‘the determination of which of two or more diseases with similar symptoms is the one from which the patient is suffering, by a systematic comparison and contrasting of the clinical findings.” (*Cooper v. Takeda Pharmaceuticals America, Inc.* (2015) 239 Cal.App.4th 555, 565, fn. 6.)

“The first step in the diagnostic process is to compile a comprehensive list of hypotheses that might explain the set of salient clinical findings under consideration. [Citation.] The issue at this point in the process is which of the competing causes are *generally* capable of causing the patient’s symptoms or

mortality. ... [¶] After the expert rules in all of the potential hypotheses that might explain a patient's symptoms, he or she must then engage in a process of elimination, eliminating hypotheses on the basis of a continuing examination of the evidence so as to reach a conclusion as to the most likely cause of the findings in that particular case." (*J&J Cases*, supra, 37 Cal.App.5th at 307, fn. 9.)

When ruling out potential causes, "an expert 'must provide reasons for rejecting alternative hypotheses 'using scientific methods and procedures' and must rely on 'more than subjective beliefs or unsupported speculation.'" (*Engilis v. Monsanto Co.* (2025) 151 F.4th 1040, 1053 citations omitted.) In other words, the expert's opinion must contain "a reasoned explanation illuminating why the facts have convinced the expert, and therefore should convince the jury, that it is *more probable than not* the negligent act was a cause-in-fact of the plaintiff's injury.'" (*Cooper v. Takeda Pharmaceuticals America, Inc.* (239 Cal.App.4th 555, 578 quoting *Jennings v. Palomar Pomerado Health Systems, Inc.* (2003) 114 Cal.App.4th 1108, 1118.)

"[A] 'differential diagnosis' can be a 'reliable method of medical diagnosis.' [citations]. But, they still must show that the steps taken as part of that analysis -- the 'ruling out' and the 'ruling in' of causes -- were accomplished utilizing scientifically valid methods." (*Milward v. Rust-Oleum Corp.* (2016) 820 F.3d 469, 476.)

Texaco argues that including gasoline/benzene exposure in the list of possible causes of Mr. Prewitt's AA was improper due, at least in part, to the time between Mr. Prewitt's exposures to gasoline/benzene and his development of AA.

Latency

Mr. Prewitt's exposures to gasoline containing benzene occurred between 1975 and 2003. He was diagnosed with AA in September 2019 – approximately 44 years after his initial exposure and 16 years after his last alleged exposure. Texaco's expert, Dr. Pyatt, cites over 60 studies indicating that when benzene exposure is sufficient enough to cause AA, the bone marrow damage becomes evidence within weeks or months after the exposure ended, and often time during the actual exposure. (D's App. p. 31-32.) As Dr. Pyatt's report notes, a recent study concluded that the "latency between exposure and hematologic manifestations ranging on average from four months to four years." (*Ibid.*) Another study involving AA induced by chloramphenicol (a potent, broad-spectrum antibiotic) showed no cases that occurred more than 1.5 years after treatment. In reviewing all of the studies, Dr. Pyatt found that they collectively "provide a consistent picture that chemically-induced AA has an onset (latency) most commonly reported in months and often while the patient is actively being treated/exposed." (*Ibid.*) The outside limit in the studies appears to be four years.

As noted above, Mr. Prewitt was first exposed to the benzene containing gasoline 44 years prior, and experienced his last exposure roughly 16 years, prior to his diagnosis. As such, based on all the studies available Dr. Pyatt concludes that the "literature describing chemically-induced AA is not reconcilable with the allegations made by Dr. Harrison in this case." (*Ibid.*; UMF 14.)

Plaintiffs 'dispute' UMF 14 by discussing studies that discuss the latency period for AML. (Resp. to UMF 14.) Plaintiffs also argues that his "diagnosis with AA fits the known latency period for AA and AML, whether that is measured from first exposure or last exposure to benzene," citing Plaintiffs' UMFs 101-109.) A review of the evidence in support of those UMFs does not aid Plaintiffs.

For example, Plaintiffs assert that “Yin 1987 showed a latency period for benzene-induced AA of 19 years.” The evidence in support, Dr. Harrison’s deposition testimony, does not corroborate this assertion. In the deposition he was asked about the Yin paper, and whether he was using that to support his opinion regarding the long latency period. This paper appears to indicate that the average latent period was “118 and a half days.” (Harrison Depo. at 144:3-16.) Dr. Harrison states:

A. Yes. [¶] And I was looking at aplastic anemia and leukemia. They were exposed to benzene from three and a half months to 19 years.

That’s not exactly the latency, right, because we don’t know the time between their exposure and when they were diagnosed. (*Id.* at 144:16-22 emphasis added.)

Plaintiffs’ UMF 103 also cites to and relies upon a couple of cases identified in the Aksoy study. To begin with, the deposition testimony in support appears to discuss a Vigliani study from 1964. (Harrison Depo. 147:20-24.) Presuming this is the same as the Aksoy study identified in the UMF, or it was misidentified, the cases cited do not appear to support Dr. Harrison’s claims. First, none of the patients were identified to have had AA. (*Ibid.*) Instead, the report related to 47 patients “two with anemia; seven had anemia and leukopenia; five anemia and thrombocytopenia; 26 had pancytopenia, six leukemia; and one a – what they called a leukemoid reaction.” (*Ibid.*)

As for the two specific cases relied upon by Dr. Harrison, cases 4 and 5, they both were eventually diagnosed with leukemia – not AA. (*Id.* at 148:23-149:10.) Likewise, the case discussed in the Degoin paper related to an eventual diagnosis of leukemia. (*Id.* at 149:23-151:7.) It appears to show that the patient had low blood counts for 13 years while he worked with paint thinners with benzene. While there was no actual diagnosis of AA, Dr. Harrison opined what they were describing was AA. The discussion appears to show that the patient had AA while he was using the benzene containing product. There is no indication there was a 13 year (or any gap) between exposure and diagnosis. The gap in exposure and diagnosis related to leukemia – which is a blood cancer and not AA.

The other studies discussed relate to AML diagnoses. There are no studies cited or relied upon by Dr. Harrison that directly state that AA can have a latency beyond that outlined by Dr. Pyatt. In addition, other than explaining that he believes that AA can sometimes (in a “fairly small percent” of cases) transition to AML, he does not explain why those studies would support the Plaintiffs’ case – since it is undisputed that Mr. Prewitt does not have AML. (UMF 9.)

The Court must “determine whether the matter relied on can provide a reasonable basis for the opinion or whether the opinion is based on a leap of logic or conjecture.” (*Sargon*, supra, 55 Cal.4th at 772.) Where the opinion is “based on a leap of logic or conjecture, the opinion may be excluded.” (*Apple Inc. v. Superior Court* (2018) 19 Cal.App.5th 1101, 1121.)

Based on the above, Dr. Harrison has not provided a sufficient explanation as to why gasoline/benzene exposure should have been included in his differential diagnosis given the lack of any scientific support for Plaintiffs’ case arising so long after his exposure ended.

Design and Manufacturing Defect

As the Court has previously ruled, Plaintiffs’ strict liability claims for alleged design and manufacturing defects are governed by Alabama law. As such, they are governed by the Alabama Extended Manufacturer’s Liability Doctrine (“AEMLD”). “The AEMLD is a judicially developed products liability

doctrine [which] is a modified version of strict liability that premises liability upon the sale by a manufacturer of a defective product.” (*Gore v. Winnebago Industries, Inc.* (11th Cir. 1992) 958 F.2d 1537, 1541.)

“To establish liability, a plaintiff must show: (1) he suffered injury or damages to himself or his property by one who sells a product in a defective condition unreasonably dangerous to the plaintiff as the ultimate user or consumer, if (a) the seller is engaged in the business of selling such a product, and (b) it is expected to and does reach the user or consumer without substantial change in the condition in which it is sold.” (*Yamaha Motor Co. v. Thornton* (Ala. Sup. Ct. 1991) 579 So.2d 619, 621 quoting *Casrell v Altec Indus.* (Ala. Sup. Ct. 1976) 335 So.2d 128, 132-33.)

“In an AEMLD action, ‘the plaintiff must affirmatively show that the product was sold with a defect or in a defective condition.’” (*Tanksley v. ProSoft Automation, Inc.* (Ala. Sup. Ct. 2007) 982 So.2d 1046, 1051 quoting *Jordan v. General Motors Corp.* (Ala. Sup. Ct. 1991) 581 So.2d 835, 836-37.) “Without evidence to support the conclusion that the product was defective and/or unreasonably dangerous when it left the hands of the seller, the burden is not sustained.” (*Ibid.*) “Proof of an accident and injury is not in itself sufficient to establish liability under the AEMLD; a defect in the product must be affirmatively shown.” (*Ibid.*)

“[O]rdinarily, expert testimony is required [in AEMLD cases] because of the complex and technical nature of the commodity.” (*Sears, Roebuck & Co. v. Haven Hills Farm, Inc.* (Ala. Sup. Ct. 1981) 395 So.2d 991, 995; see also, (*Parker v. Tricam Indus.* (M.D.Ala. May 1, 2025, No. 3:24-cv-79-RAH) 2025 U.S.Dist.LEXIS 82983, at *23 [“Alabama courts consistently require expert testimony to prove a defect [because] lay juries lack the knowledge, training, and expertise necessary to determine the existence of a defect in a technical product.”].) “When the product in question is of a complex and technical nature such that a lay juror could not, in the absence of expert testimony, infer that a defective condition of the product caused the product's failure and caused the resulting injury to the plaintiff, expert testimony is a necessary component of a plaintiff's case.” (*Cooper v. Toshiba Home Tech Corp.* (Ala. Mid. Dist. 1999) 76 F.Supp.2d 1269, 1276, multiple citations omitted.)

With a design defect claim, “a plaintiff must prove that ‘a safer practical, alternative design was available to the manufacturer at the time it manufactured the [product]. The existence of a safer, practical, alternative design must be proved by showing that:

(a) The plaintiff's injuries would have been eliminated or in some way reduced by use of the alternative design; and that

(b) taking into consideration such factors as the intended use of the [product], its styling, cost, and desirability, its safety aspects, the foreseeability of the particular accident, the likelihood of injury, and the probable seriousness of the injury if that accident occurred, the obviousness of the defect, and the manufacturer's ability to eliminate the defect, the utility of the alternative design outweighed the utility of the design actually used.” (*GMC v. Jernigan* (Ala. Sup. Ct. 2003) 883 So.2d 646, 662 citation omitted.)

“Thus, to testify that a challenged product is defectively designed, an expert witness must be qualified to testify that ‘a safer, practical, alternative design was available to the manufacturer.’” (*Beam v. McNeilus Truck & Mfg., Inc.* (Ala. N. Dist. Ct.) 697 F.Supp.2d 1267, 1276 quoting *GMC*, supra, 883 So.2d at 662.) The expert must also be “qualified to testify that the utility of the alternative design

outweighs the utility of the challenged design, and that use of an alternative design would have eliminated or reduced the injuries suffered.” (*Ibid.* citations omitted.) The plaintiff must also submit evidence “from which the jury could find the suggested alternatives are not only technically feasible but also practicable in terms of costs and the over-all design and operation of the project.” (*Elliott v. Brunswick Corp.* (11th Cir. 1990) 903 F.2d 1505, 1510.)

Texaco first argues the Plaintiffs have failed to designate an expert to testify regarding the design and manufacturing defect issues. Texaco cites Plaintiffs’ interrogatory responses which indicate that Plaintiffs intended to provide Dr. Harrison and Dr. Jones as experts regarding the design and manufacturing defects. (App. Evid. at e.g. 327 (No. 43); 334 (No. 48).)

Plaintiffs’ expert disclosure form, however, does not designate either of those persons, nor anyone else, as an expert pertaining to design and/or manufacturing defects. (App. Evid. at 276-79.) The Disclosure lists several medical providers that may testify as to Mr. Prewitt’s medical condition as well as two experts “designate[d] as causation expert witnesses,” *i.e.* Drs. Harrison and Jones. (App. Evid. at 276-79.) There is no indication that Plaintiffs have designated any expert regarding design or manufacturing defects.

Plaintiffs do not address this concern directly, but instead ‘deny’ the relevant UMFs and indicate that Dr. Jones’ report provides evidence in support of manufacturing and design defects, citing particular portions of her report. (See UMF Response 17-21.) Plaintiffs argue that Dr. Jones’ report shows that gasoline with lower levels of benzene than that included in Texaco’s gasoline was available on the market as early as the 1970’s. (Opp. at 22:18-26.) They argue that lower benzene levels in gasoline is safer than gasoline with more benzene because it causes less exposure to benzene. While this explanation may make sense in the abstract, Plaintiffs experts fail to provide any opinions supporting, explaining, or qualifying this theory. In addition, Plaintiffs fail to point to any testimony or evidence which suggests that the alternative lower benzene gasoline formula is “not only technically feasible but also practicable in terms of costs and the over-all design and operation of the project.” (*Elliott*, *supra*, 903 F.2d at 1510.)

To the extent Plaintiffs attempt to rely on the US EPA report, such a report is not a substitute for expert testimony discussing the scientific principles and methodologies at issue and explaining these complicated issues to the trier of fact. Lay jurors would have no way to decipher the 140+ page technical paper, much less how its technical finds relate to this case. For example, the paper discusses benzene related to vehicle exhaust – not exposure via inhaling gasoline fumes – and its impact on cancer rates. It is undisputed, however, that Mr. Prewitt’s AA is not cancer. As such, further explanation would be required.

Based on the above, Texaco’s MSA as to the design defect and manufacturing defect causes of action is **granted**.

Summary and Conclusion

Texaco has met its burden to show that Plaintiffs’ causes of action have no merit by showing that the element of causation cannot be established. As such, the burden shifted to Plaintiffs to show that a triable issue of material facts does exist. “To meet this burden, the plaintiff must produce admissible evidence.” (*Borders Online*, *supra*, 129 Cal.App.4th at 1188, citations omitted.) This,

Plaintiffs have failed to do.

Texaco has also shown that Plaintiffs do not have admissible expert testimony required to support their defect cause of action.

Accordingly, Defendant Texaco's motion for summary judgment is **granted**.

Evidentiary Issues

Objections to Evidence

Plaintiffs' Objections

Written objections to evidence must follow one of two formats as set forth in the California Rules of Court. (Cal. R. Ct. 3.1354(b)). The objections must be "numbered consecutively" and must (1) identify the name of the document where the specific material objected to is located, (2) state the exhibit, title, page, and line number of the material objected to, (3) quote or set forth the objectionable statement or material, and (4) state the ground for the objection. (*Ibid.*)

Plaintiffs do not number any of their objections, and while the first two documents they are objecting to are identified, the third objection is to a statement – but Plaintiffs do not identify the name of the document where that statement is located, or the exhibit, title, page and line number where it can be found. Without this information, the Court cannot understand the context of the statement or even who made it.

The party submitting the objections "must submit with the objections a proposed order." (*Ibid.*) The proposed order must include a place for the Court to indicating whether it has sustained or overruled each objection, and a place for the signature of the judge. (*Ibid.*) The proposed order must also be in one of two formats. (*Ibid.*) The failure to comply with the requirements of Rule 3.1354 is sufficient basis to overrule those objections. (*Schmidt v. Citibank, N.A.* (2018) 28 Cal.App.5th 1109, 1118.)

Plaintiff's objections fail to comply with the above requirements and can consequently be overruled on that basis alone. The Court also finds each of Plaintiffs' objections to be without merit and overrules each accordingly.

Texaco's Objections

Objections 1-10: Sustained (Improper opinion *Jacobs v. Fire Ins. Exchange* (1995) 36 Cal.App.4th 1258, 1270.)

Objection 11: Sustained (Evid. Code § 1200.)

2. 9:00 AM CASE NUMBER: C23-00639
CASE NAME: 4740 APPIAN WAY HOMEOWNERS ASSOCIATION VS. CONTRA COSTA COUNTY
HEARING ON DEMURRER TO: 1ST AMENDED COMPLAINT
FILED BY: CHANG, HOUY CHYUN
TENTATIVE RULING: